



**25TH IUHPE WORLD CONFERENCE
ON HEALTH PROMOTION**
SETTINGS FOR PLANETARY HEALTH & WELL-BEING
13 – 16 MAY 2025 | ADNEC CENTRE ABU DHABI

دائرة الصحة
DEPARTMENT OF HEALTH

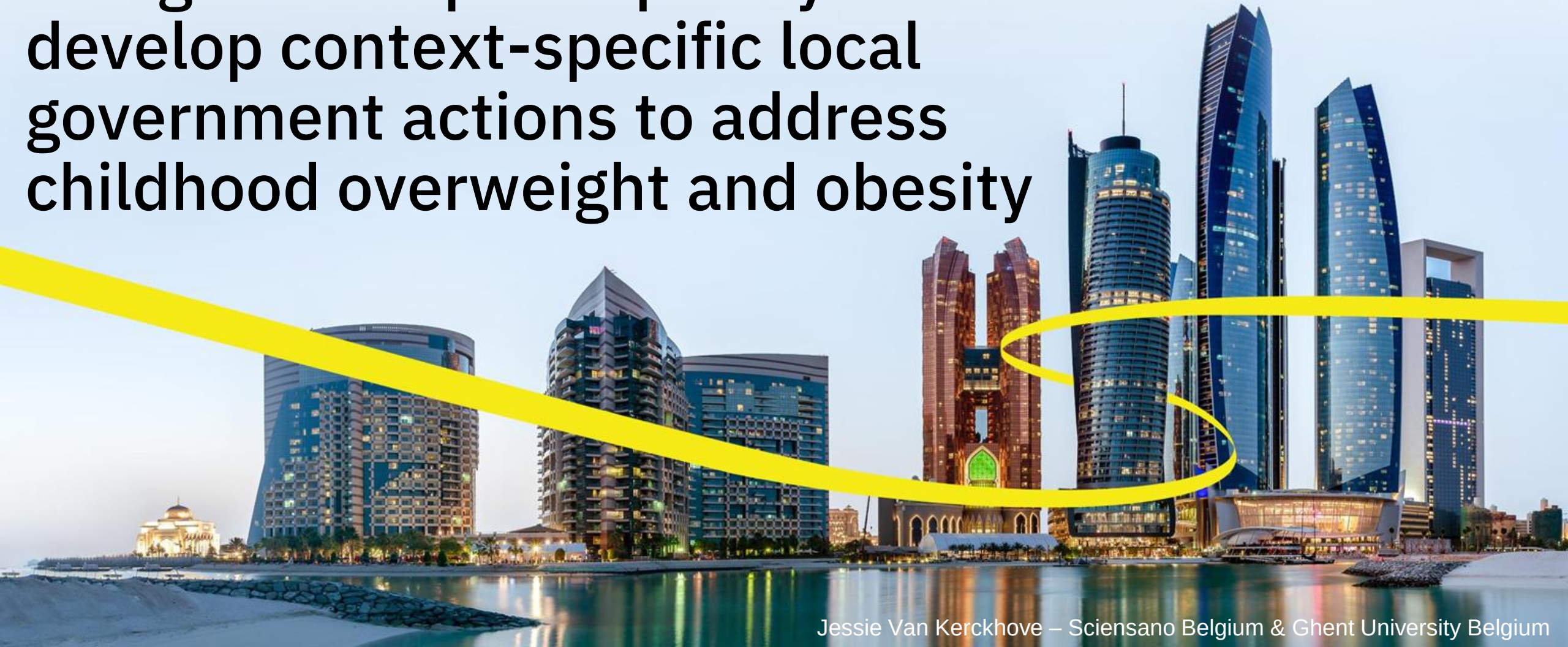


مركز أبوظبي
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Using diverse participatory methods to develop context-specific local government actions to address childhood overweight and obesity





Context

Health4EuKids: *a joint action for the implementation of best practices and research results on Healthy Lifestyle for the health promotion and prevention of non-communicable diseases and risk factors*



Best practices in action – Grünau Moves

How to

- transfer,
- implement,
- and sustain

effective health promotion interventions for children aged 4-12

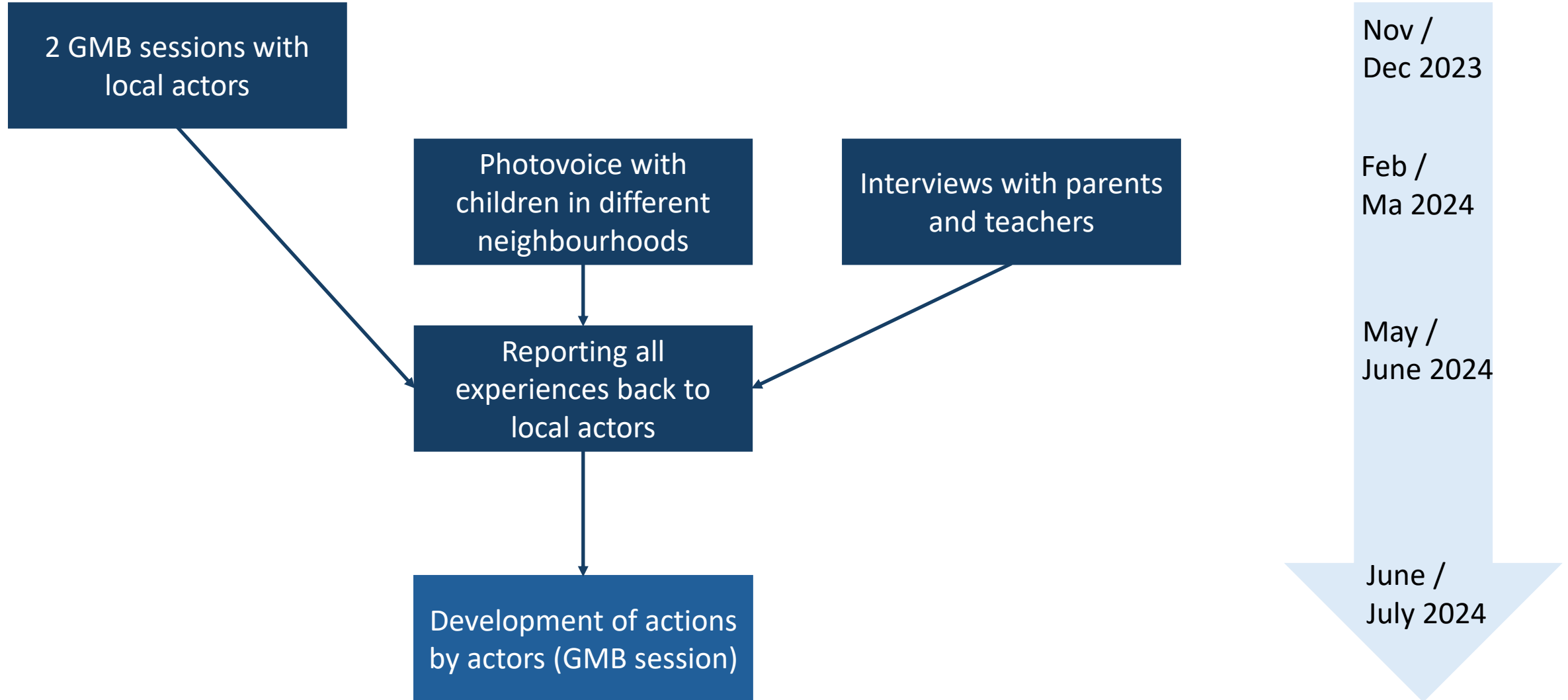
Case study: Flanders (Belgium)

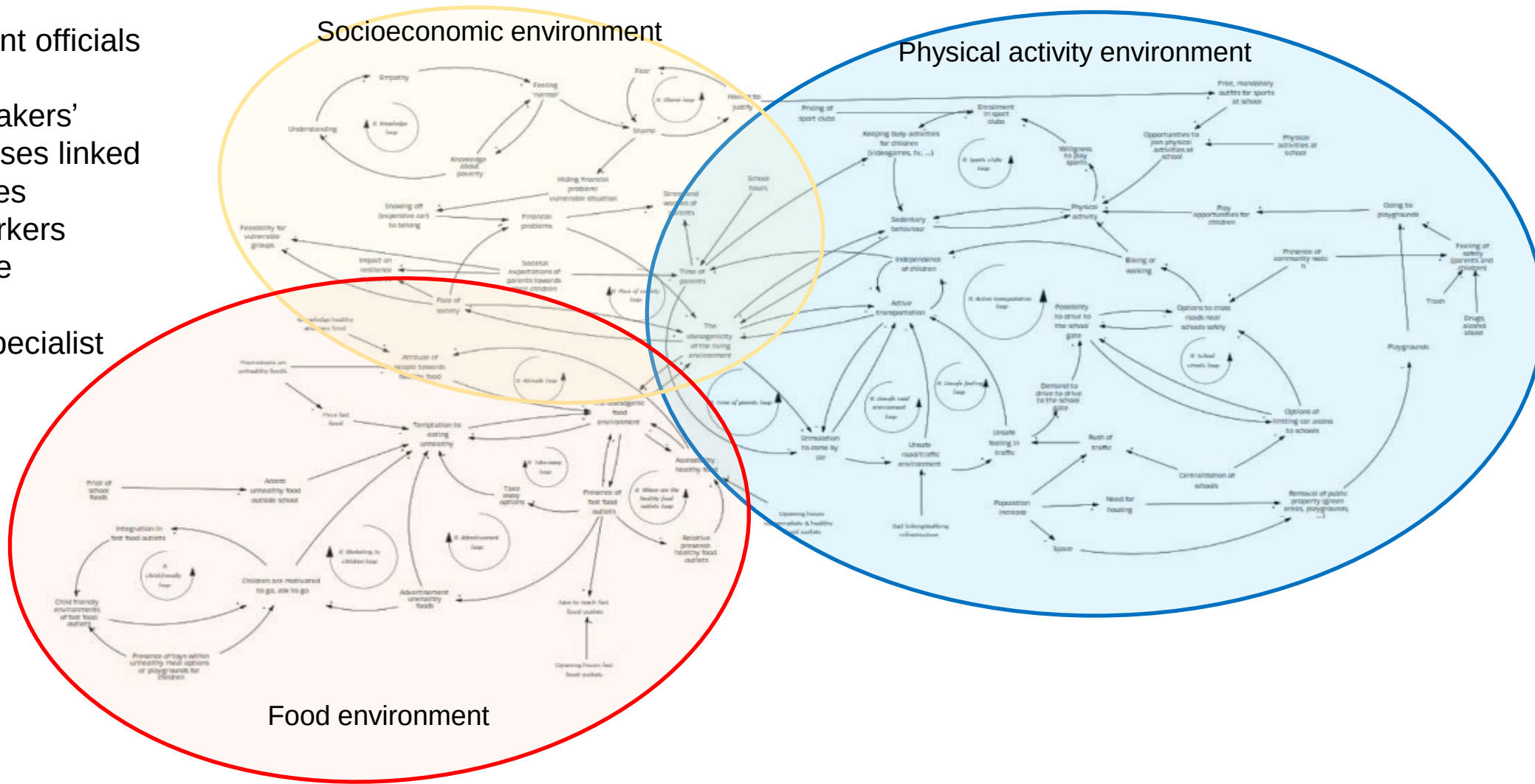
Maasmechelen

Eeklo



Methodology needs assessment







Part 2: Photovoice & interviews

OUR CHILDREN

- 4 classes of a city centre school (ages 6-8)
- 1 class in a remote neighbourhood (ages 5-6)
- Children from a youth organization, specifically working with children living in vulnerable situations
- Total = 90 children



Part 3: Interviews with parents and teachers

Physical activity environment

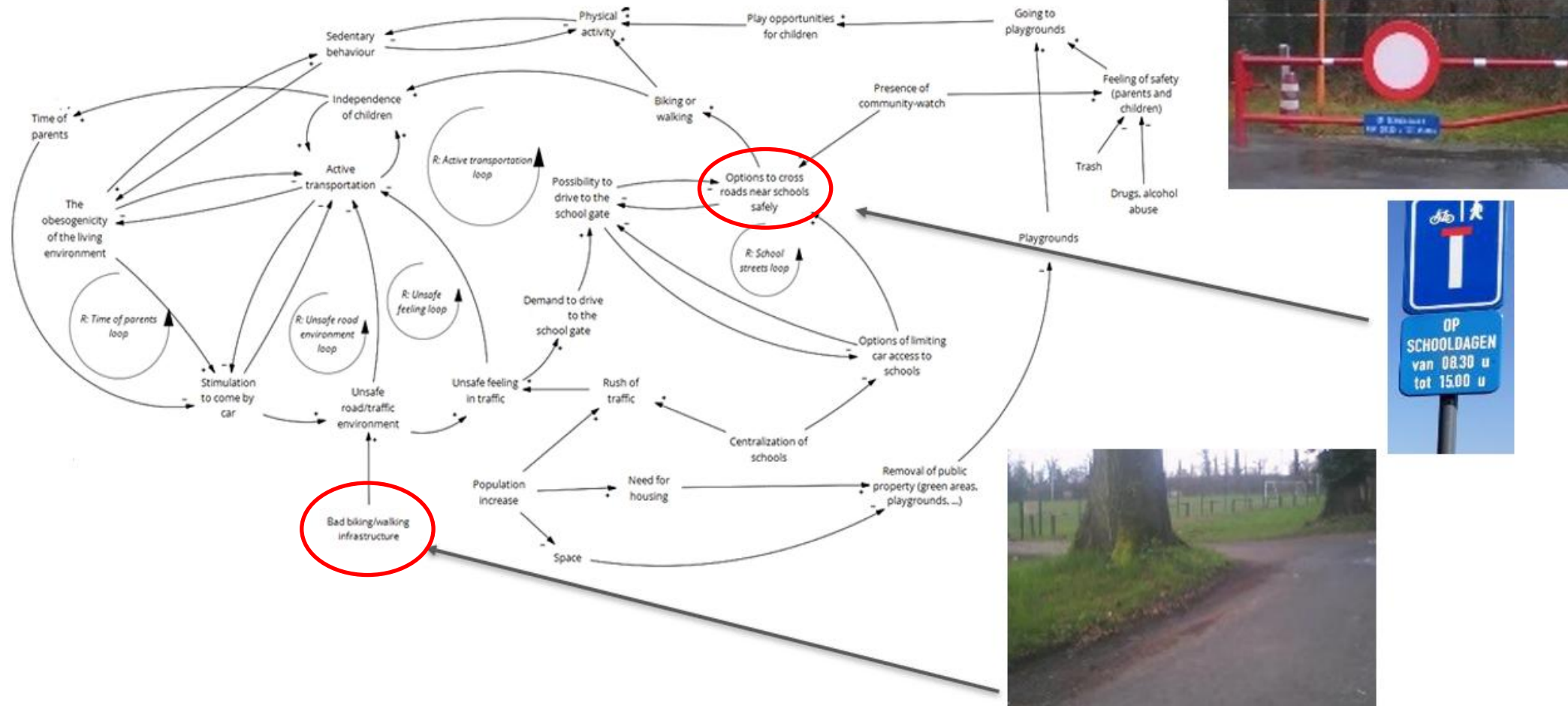
- Focus on road safety
- Focus on opportunities for play
- Focus on (limitations) of school environments

Food environment

- Focus on lack of healthy options throughout the municipality
- Focus on unhealthy or empty lunch boxes
- Focus on parent resistance



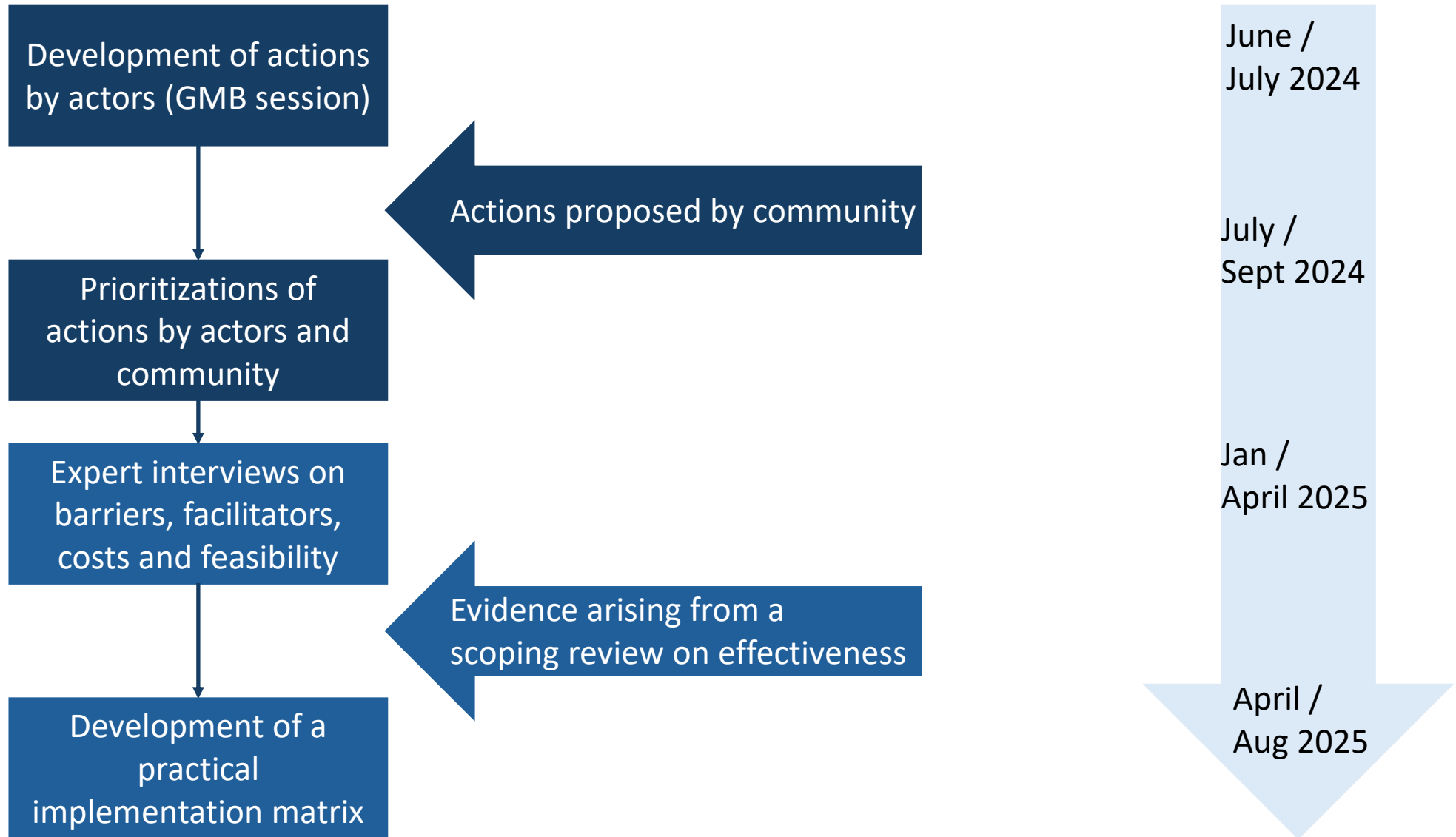
Part 4: Shared understanding of the barriers



Part 5: Development of actions

Theme	Possible places to intervene	Proposed actions	Support
Food and the food environment at school	(V) Accessibility healthy food (L) <i>Where are the healthy food outlets</i> (V) Price of school foods	- Offer hot (healthy) meals at school - Offer free fruit and/or soup at school - Healthy breakfast at school	- Actors: 20% - Actors 70% - Actors 30%
Active to school	(V) Active transportation (L) <i>Active transportation loop</i> (V) Shame (L) <i>Shame loop</i> (V) Hiding financial problems	- Create a bus service for walking and/or biking - Frequent organization of “bike and walk” days to school - Offer biking lessons for everyone - Offer bikes at low prices for people of low SES - Organization of a second-hand fair with the aim of sustainability rather than cheap prices	- Actors: 40% - Actors: 60% - Actors: 90% - Actors: 90% - Actors: 60%
Communication	(V) Feeling of safety (parents and children)	- Better use and connection with the neighbourhood policy officer - Increase the amount of ‘points of contact’ throughout the city - More community consultation and direct communication between the local government and its members	- Actors: 50% - Actors: 30% - Actors: 20%

Next steps: implementation



Reflection

Strengths

- Interdisciplinary health network
- Inclusion of children of different ages and backgrounds
- Intercept interviews to engage parents
- Researcher as independent factor between community and actors

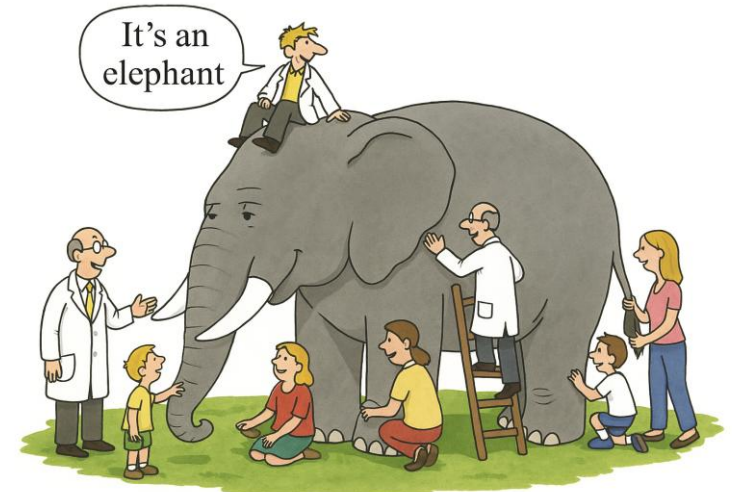
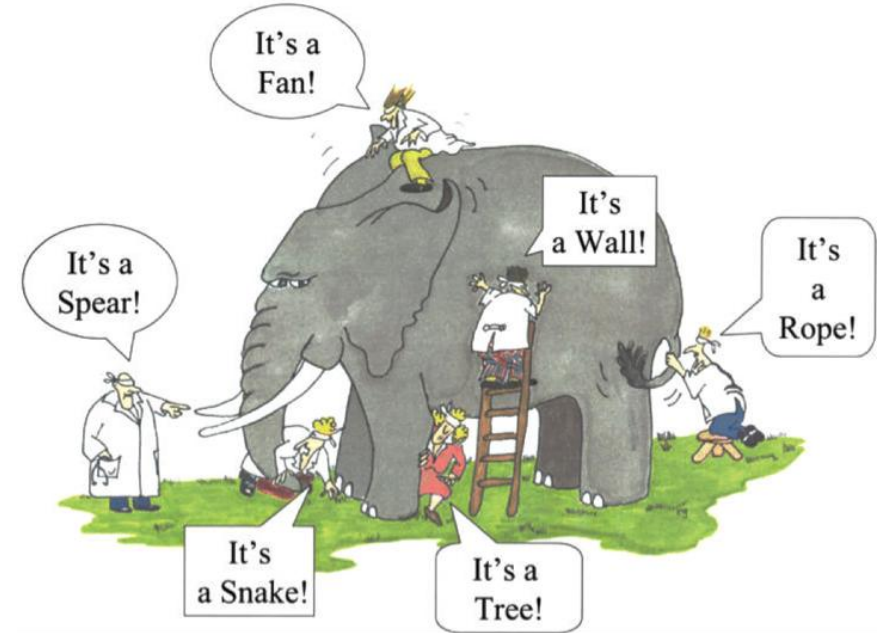
Limitations

- Organisation of photovoice with schools
= focus mainly on school environment
- Recruitment of children experiencing vulnerable situations = difficult
=> use of trusted social workers (inclusion of their opinion?)
- Limited time with actors



Conclusion

- Combination of age-appropriate methods
- Highlighted barriers from different points of view
- Created understanding
- Offering an example for other local governments





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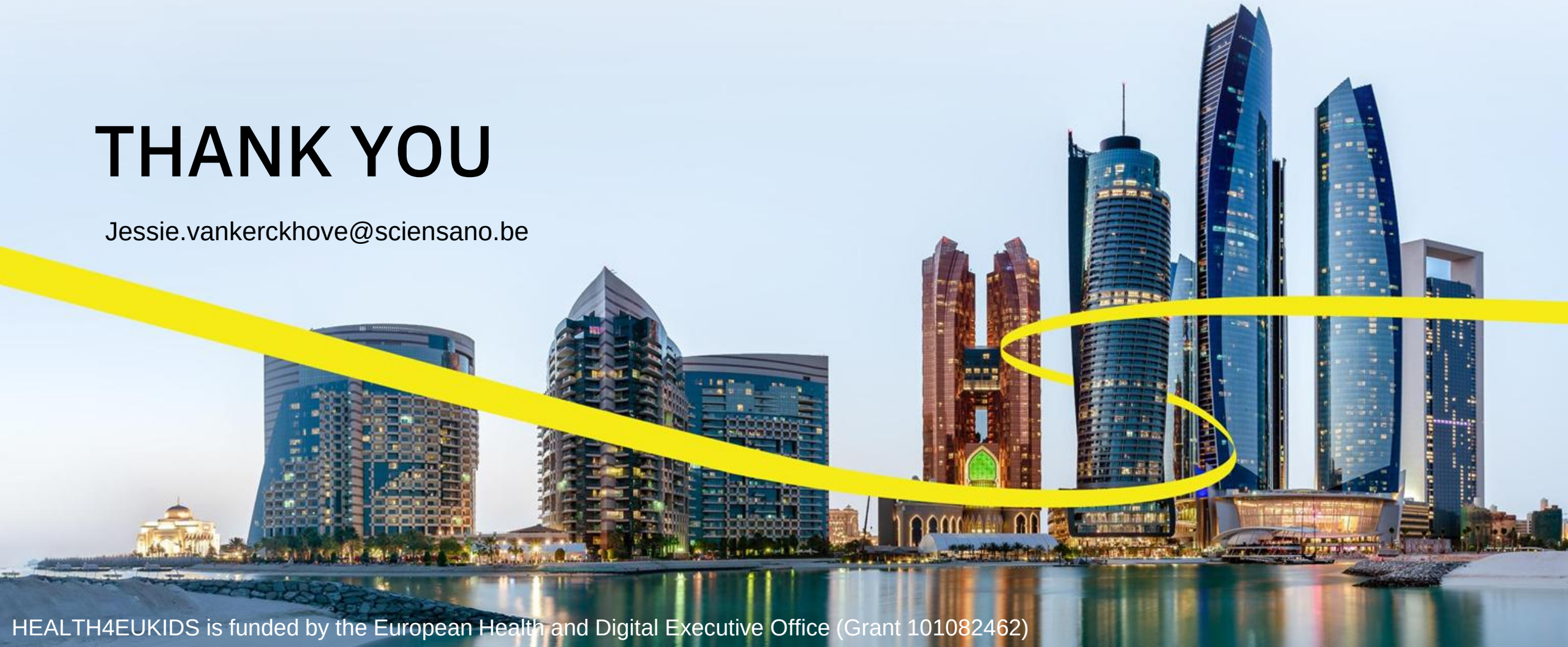
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THANK YOU

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